

Patient ID: WheelerPeriod: April 1 to April 30, 2026Report Date: May 1, 2026Coverage: 240/720h (33%)

Decision-support summary derived from longitudinal vital sign trends; interpret in clinical context.

Clinical Alerting Thresholds

Very Low HR < 40 bpm

Very High HR > 110 bpm

Low HR < 45 bpm

Elevated Breathing > 24 brpm

Elevated HR > 95 bpm

High Breathing > 30 brpm

High HR > 100 bpm

Very High Breathing > 40 brpm

Patient clinical status over 30 days from Apr 01 to Apr 30

HR Breathing

Elevated Breathing

Apr 01 to Apr 03 (3d)

Elevated Breathing

Apr 15 to Apr 30 (9d)

High Breathing

Apr 27 to Apr 27 (1d)

Last 24 Hours

Last 24 Hours: RED

30-Day Tier: RED

Vital	Avg	Min	Max
Heart Rate	64 bpm	51 bpm	80 bpm
Breathing	24 brpm	8 brpm	34 brpm

Last 24 hours: two elevated breathing episodes overnight, otherwise within normal range.

Episodic Events (last 24h)

Date	Time Span	Duration	Episodes	Condition	Avg	Min	Max	Comment
Apr 30	3 AM to 4 PM	14h	2	Elevated Breathing	26	10	34	Check SpO2 and lung sounds; assess for fluid overload or early infection

28 episodic events Detected, spanning 3 days total. Conditions: Elevated Breathing, High Breathing. Priority grade: High

Episodic Events	Total Hours	Highest Burden	Episodes/day	Coverage
28	92h	Elevated Breathing	<1	33%

Major Findings: Sustained elevated breathing (avg 26, peak 48)

Date	Time Span	Duration	Episodes	Condition	Avg	Min	Max	Comment
Apr 27	8 AM to 9 AM	1h	1	High Breathing	30	26	33	Check SpO2, work of breathing, fever; consider pulmonary cause or CHF
Apr 01	1 AM to 3 PM	32h	8	Elevated Breathing	26	9	48	Check SpO2 and lung sounds; assess for fluid overload or early infection
Apr 15	9 PM to 10 PM	59h	19	Elevated Breathing	26	9	47	Check SpO2 and lung sounds; assess for fluid overload or early infection

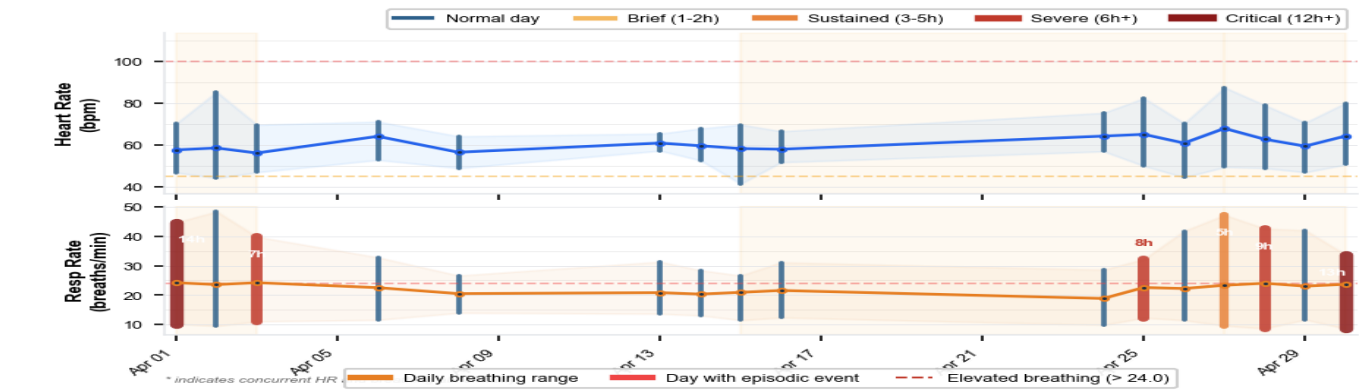
Low data coverage (33%); interpret with caution.

Suggested Clinical Review Actions

- Elevated Breathing (avg 26 brpm, peak 47 brpm): Check SpO2 and lung sounds; assess for fluid overload or early infection
- High Breathing (avg 30 brpm, peak 33 brpm): Check SpO2, work of breathing, fever; consider pulmonary cause or CHF
- Overall trajectory shows 3 unstable phases with 19 bpm variability. Consider increasing monitoring frequency and lowering escalation threshold.

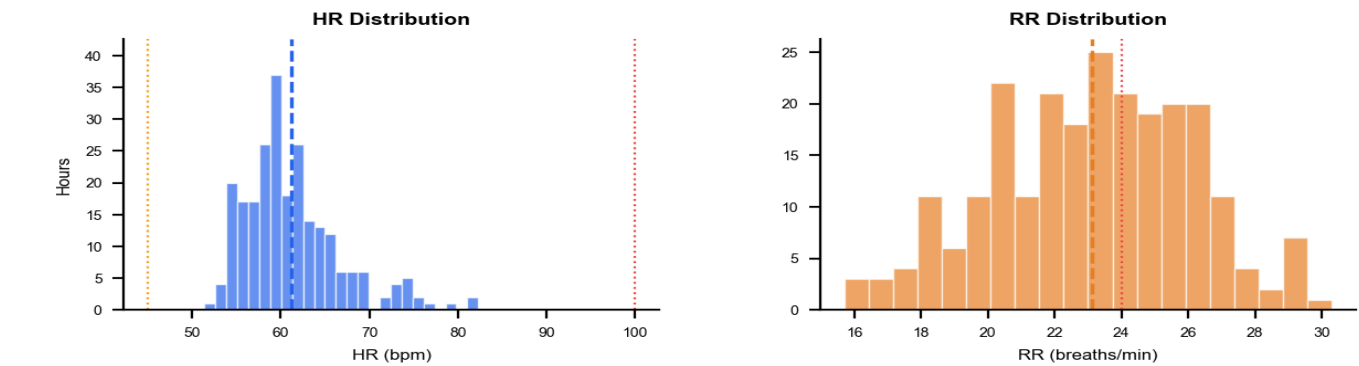
Clinical Pattern Observations:

- 1. Sustained finding: 14h continuous Elevated Breathing on Apr 01.
- 2. Clustered pattern: Episodic events on 6 of 30 days (20%).
- 3. Nocturnal pattern: 3 of 4 eligible episodes during evening or night hours.

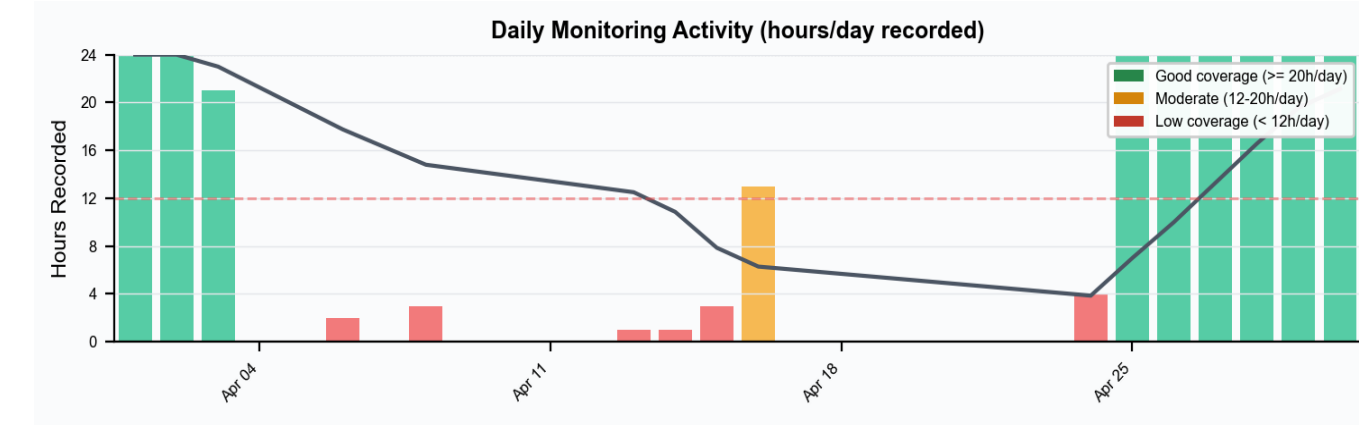


Red bars indicate days with detected episodic events.

Vital Sign Distribution



Monitoring Activity



Bars indicate monitoring hours/day. Red threshold line indicates clinical monitoring baseline.

■ HR episode ■ RR episode ■ Recorded, no episode ■ No data

Episodic event: threshold exceeded continuously for ≥ 1 hour. Episodes separated by ≤ 1 hour of normal vitals are counted as the same event. Brief threshold crossings that do not sustain are not flagged.